



March 15, 2024

White House Office of Science & Technology Policy
National Coordination Office
Networking and Information Technology Research and Development Program
2415 Eisenhower Avenue
Alexandria, VA 22314
Attn: Faisal D'Souza, NCO

Submitted via email at [REDACTED]

Re: AI Action Plan

On behalf of the AI Healthcare Coalition, thank you for the opportunity to contribute to the development of the United States Artificial Intelligence (AI) Action Plan to advance America's AI leadership. We are pleased to submit our response to the White House Office of Science & Technology Policy (OSTP) and the National Coordination Office (NCO), Networking and Information Technology Research and Development (NITRD) Program. We are committing to assisting the Trump Administration in its execution of President Trump's Executive Order (EO) 14179, "Removing Barriers to American Leadership in Artificial Intelligence," and look forward to working with you on issues concerning the intersection of health and technology policy.

Founded in 2019, the AI Healthcare Coalition convenes healthcare AI innovators and stakeholders to advocate for patient access to safe, ethically-developed healthcare AI. Our members are AI innovators who have, or are in the process of developing, AI services (or devices) that require U.S. Food & Drug Administration (FDA) oversight and market authorization. Our founding membership includes the first-ever autonomous AI innovator to obtain FDA authorization, as well as innovators who have obtained the first-ever Medicare reimbursement for their respective AI services.

As you develop our national AI Action Plan, we offer these recommendations for your consideration, and hope to be a continuing resource as you consider the complexities of AI deployment in healthcare settings. We offer our thoughts below and look forward to continuing the conversation.

I. Executive Actions to Unleash AI & Health Innovation

The Trump Administration can take clear steps now to ensure that FDA-authorized, rigorously validated AI health systems reach patients and this American industry sector flourishes. We offer below specific policy recommendations and background.

U.S. Food & Drug Administration (FDA)

- Avoid Duplicative Regulatory Burden for FDA-authorized AI Models. Rapid innovation in medical technology, including AI, has the potential to Make Americans Healthy Again by addressing chronic disease and increasing efficiencies. Rigorous safety validation by the US Food & Drug Administration (FDA) is an essential component to ensure trust in such innovation. The FDA has done significant work to evaluate and approve certain AI/ ML-enabled medical devices. The FDA has also released numerous guidance documents concerning AI/ML oversight, including the *AI/ML-Based Software as a Medical Device (SaMD) Action Plan*,¹ the *Proposed Regulatory Framework for Modifications to AI/ML-Based SaMD*,² and the *Clinical Decision Support Final Guidance*,³ and the *Marketing Submission Recommendations for a Predetermined Change Control Plan for AI/ML-Enabled Device Software Functions*.⁴

Given the volume of already-existing regulation specific to health AI systems, it is paramount that the U.S. does not impose duplicative, industry-agnostic laws and regulations that are overly burdensome for American AI healthcare innovation, particularly since many AI healthcare products are already subject to extensive regulatory review. It is imperative to foster a legal and regulatory schema for healthcare AI that puts American companies in a position where they can compete, develop, use, and commercialize AI.

We caution against other regions' industry agnostic, top-down approach to AI regulation, as we have seen some AI innovator companies cease operations in

¹ U.S. Food & Drug Administration (FDA), "Artificial Intelligence/Machine Learning (AI/ML)-Based Software as a Medical Device (SaMD) Action Plan," (January 2021) available at www.fda.gov/media/145022/download?attachment.

² FDA, "Proposed Regulatory Framework for Modifications to Artificial Intelligence/Machine Learning (AI/ML)-Based Software as a Medical Device (SaMD): Discussion Paper and Request for Feedback," (April 2019) available at <https://www.fda.gov/media/122535/download?attachment>.

³ FDA, "Clinical Decision Support Software: Guidance for Industry and Food and Drug Administration Staff," (September 2022) available at <https://www.fda.gov/regulatory-information/search-fda-guidance-documents/clinical-decision-support-software>.

⁴ FDA, "Marketing Submission Recommendations for a Predetermined Change Control Plan for Artificial Intelligence-Enabled Device Software Functions," (December 2024) available at <https://www.fda.gov/regulatory-information/search-fda-guidance-documents/marketing-submission-recommendations-predetermined-change-control-plan-artificial-intelligence>.

regions where such approaches have been implemented. AI innovation and deployment is a global endeavor, and our U.S. companies should be set up for successful development and provision of safe, trustworthy AI. Like we have seen in recent U.S. proposals to address supply chain disruptions, once a market becomes untenable for manufacturing and production, it is very difficult to bring essential capabilities back to U.S. soil. Ensuring that we have the capabilities here at home to provide the best care—through AI, tech capabilities, and common-sense approaches to AI safety and ethics—is essential to American competitiveness in global AI innovation.

- Facilitate Public-Private Standards Development. We increasingly hear that the regulatory processes available to the FDA can be opaque for innovator companies seeking review, and stakeholders have observed that the process can at times be ad hoc, unpredictable, and slow—thwarting innovation and adoption of revolutionary technologies. To address these issues and hasten review for innovative products, the FDA has created and implemented the ‘Collaborative Community’ methodology. This framework is meant to create broad, international stakeholder consensus from patients, physicians, scientists, ethicists, payors, technology creators and investors.

More than twenty-two of such Collaborative Communities have been created to date. In one example, the Collaborative Community on Ophthalmic Innovation (CCOI), was founded in 2021 in coordination with the FDA with over 100 widely respected AI, clinical and methodological experts from academia and industry. CCOI now has over 1000 contributing members, and the workgroup created the first ever consensus statement, “Foundational Principles of AI”, which outlined AI trust evaluation criteria based in ethics and patient benefit. This work was developed in a public, transparent process over multiple years, culminating in publication in 2022 with multiple FDA and academic co-authors.

CCOI’s “Foundational Principles of AI” represents a practical effort by the FDA to nimbly develop standards that can be utilized to engender more dynamic review of emerging technologies like AI. We urge you to ensure that this and similar work is leveraged by the FDA to help AI innovators obtain thorough, rigorous review of their technologies grounded in specialty-specific, consensus-driven standards.

On the clinical standards side specifically, among the standards for consideration in clinical validation are ISO-14155 clinical investigation and ISO-14971 risk management. Both standards may be applied broadly, however, industry could benefit from more specific standards that take the uniqueness of AI/ML into consideration. For example, the Association for the Advancement of Medical Instrumentation® (AAMI) developed a guidance document on the application of

ISO-14971 to AI/ML, which discusses AI/ML specific risks including data drift. Similarly, sponsors of clinical studies could benefit from AI/ML clinical standards that could take AI-specific topics into account, such as the deidentification and management of data used in clinical studies.

Successful facilitation of public-private standards will create greater transparency and consistency in regulatory review and commercialization of cutting-edge AI. Because existing requirements can be opaque and outdated as applied in the context of health AI, transparent and stakeholder-informed standardization will accelerate the development of AI health technologies.

Centers for Medicare & Medicaid Services (CMS)

- Implement President Trump's Medicare Coverage for Innovative Technologies (MCIT) Program. During President Trump's first term, the MCIT program was improved to provide a transitional Medicare coverage policy for certain medical technologies that receive "breakthrough technology" status under the FDA. President Biden pulled back this policy, and instead finalized a similar policy, entitled the "Transitional Coverage for Emerging Technologies," or TCET, program. While this program makes some positive steps forward by requiring CMS and the FDA to work contemporaneously and iteratively with innovator applicant companies, the agencies have said that very few—approximately four—applications would be eligible for the TCET process annually. This is far too few. We urge the Administration to facilitate timely coverage for AI health technologies by finalizing a MCIT-like policy. In addition to increased patient access, such a policy would create confidence for investment in these important technological advancements, aiding commercialization.
- Create Permanent Medicare Payment Pathways for AI Technologies. While some AI healthcare services are currently reimbursed under the Medicare program, there is no clear pathway for clinicians and innovators to obtain reimbursement for AI services. As more AI services are rigorously validated and FDA-authorized, a clear reimbursement pathway—or multiple pathways—is/are needed to ensure that Medicare beneficiaries and clinicians can access these services. Notably, AI services are diverse and varied, spanning multiple medical specialties and incorporating modalities that vary in their use of algorithmic design and machine learning.

For example, in the case of AI services provided in the outpatient setting, we often see such services assigned to a clinical ambulatory payment classifications (APCs) that do not adequately reimburse for provider costs because there is no specific pathway for AI services. While CMS does have a New Technology APC

process to allow reimbursement for new technologies in the outpatient setting, that process was not created to facilitate the payment of AI technologies, and often presents too short or low a payment to allow access. For these reasons, we support a 5-year outpatient payment rate for AI technologies. Permanent payment pathways will create certainty for AI technology developers and providers, which would drive investment and accelerate innovation.

- Ensure Separate Payment for AI Services and Underlying Imaging Services. CMS recently finalized a policy in its Outpatient Prospective Payment System (OPPS) rule to allow separate payment for the AI service and the underlying imaging service, but we see ongoing challenges in specialty areas where imaging services and payment are bundled (for example, AI that is viewed as computer-aided detection (CAD)). We urge the administration to provide further guidance in this area to ensure that its separate payment policy, as finalized by CMS in the CY 2023 OPPS final rule, is effectuated.
- Exclude AI Services from the OPPS Cap on Imaging Services. Sec. 5102(b) of the Deficit Reduction Act (DRA) requires that “for imaging services [...] furnished on or after January 1, 2007, [CMS] will cap the [technical component] TC of the PFS payment amount for the year (prior to geographic adjustment) by the [annual] OPPS payment amount (prior to geographic adjustment).” Congress enacted Section 5102(b), also known as the OPPS cap, to address rapid growth in spending on imaging services, particularly for advanced imaging modalities—such as computed tomography (CT), magnetic resonance imaging (MRI), and nuclear medicine—as compared to growth in spending among less advanced imaging modalities, such as x-ray or ultrasound.

As is clear in the statutory text, the DRA envisioned services such as x-rays, ultrasound, PET, MRI, and CT as such “imaging services,” not autonomous AI or other Software as a Service (SaaS) technologies. Further, when CMS implemented the OPPS cap pursuant to Sec. 5102(b), CMS expressly excluded some services because those services encompassed more than the use of imaging technology alone. CMS should recognize an additional exclusion for services for TC-only services for which there is no associated PC, as CMS has previously recognized that codes without a TC / PC split should not be listed on the OPPS Cap List.

- Create a Pathway for Permanent, Separate Payment for AI services whose NTAP Rate has Expired. Similar to CMS’ New Tech APC program, inpatient AI services may apply for and receive an New Technology Add-on Payment (NTAP) payment for 2-3 years if they meet certain criteria. Once NTAP expires, CMS has reasoned that the cost of the technology should be covered by the DRG payment. The

industry does not have data as to whether or not this is happening, and we believe that a lack of ongoing separate payment leads to a lack of provider adoption / patient access. This is particularly true given recent CMS proposals that may reduce the time that NTAP status actually applies to a new technology in practice. The ambiguity about reimbursement post-NTAP disincentivizes investment in these tools which can benefit the Medicare program. If appropriately reimbursed, such tools are likely to be adopted by healthcare providers, improve patient care, and create efficiencies.

U.S. Department of Health & Human Services Assistant Secretary of Technology Policy (HHS/ASTP)

- Exclude FDA-authorized AI systems from HHS/ASTP (legacy Office of the National Coordinator (ONC) from burdensome “predictive decision support intervention (DSI)” regulations for certified electronic health record (EHR) vendors. These rules, which were promulgated under President Biden’s term, require onerous reporting by EHR vendors—and in turn by those AI systems that run on such vendors platforms. In a May 2024 fact sheet posted by the agency, HHS/ASTP stated that these “requirements are intended to provide users and the public greater information on whether a Predictive DSI is fair, appropriate, valid, effective, and safe (what we refer to as the FAVES quality framework).” Further, HHS/OSTP opined:

“The anticipated outcome of such transparency will increase public trust and confidence in Predictive DSIs, allowing users including healthcare systems, clinicians, and patients, to expand the use of these technologies in safer, more appropriate, and more equitable ways. The information this requirement makes available will support users in navigating the market for predictive and generative AI in healthcare. It will also help address numerous challenges, including risks of bias and harm, that such tools can present. This set of information will become a consistently available, industry-wide baseline upon which others can build, standardize, and enhance.”⁵

These grounds are inconsistent with President Trump’s EO 14179, “Removing Barriers to American Leadership in Artificial Intelligence,” as they present additional regulatory burden for AI developers. Further, they raise the specter of increased compliance liability for both providers and developers, thwarting

⁵ U.S. Department of Health & Human Services, Office of National Coordinator. “ONC Health IT Certification Program Resource Guide: Decision Support Interventions Certification Criterion (45 CFR 170.315(b)(11))” (May 2024) available at https://www.healthit.gov/sites/default/files/page/2024-05/DSI-Criterion-Resource-Guide_508.pdf

adoption. At a minimum, for FDA-authorized AI/ML medical devices, ONC should permit reference to FDA submission and approval documents in lieu of satisfaction of the predictive DSI source attribute reporting.

U.S. Department of Health & Human Services Office of Civil Rights (HHS/OCR)

- Exclude FDA-authorized AI systems from HHS/OCR's Sec. 1557 Regulations. On May 6, 2024, HHS / OCR and CMS finalized updates to regulations implementing Section 1557 of the Affordable Care Act (ACA) to increase anti-discrimination protections for covered health care entities.⁶ Among other provisions, this rule established new obligations for providers who utilize artificial intelligence (AI) and other patient care decision support tools. At the time of issuance, HHS noted that this regulation was in furtherance of President Biden's now-rescinded EO on "Safe, Secure, and Trustworthy Development and Use of Artificial Intelligence."

Specifically, the Sec. 1557 final rule applies nondiscrimination principles to the use of new and broadly defined "patient care decision support tools" in clinical care, and requires those subject to the rule to identify and mitigate discrimination when they use patient decision support tools, including automated and non-automated tools, mechanisms, methods, and technology to provide patient care. As of now, covered entities are required to comply with the new patient care decision support tool requirements, including an ongoing monitoring and risk mitigation responsibilities.

Although stakeholders expressed concerns regarding regulatory duplication at the time that the rule was proposed, OCR expressly declined to exclude FDA-authorized technologies from the scope of the rule. Importantly, while ONC's requirements for predictive DSIs apply to health information technology developers, Section 1557's requirements apply to covered entity users of patient care decision support tools. These requirements create a redundant regulatory burden for AI innovator companies and healthcare providers, especially for those tools that are already subject to FDA review and authorization.

II. Legislative Actions to Empower AI & Health

Federal legislative changes are also needed to ensure that American health AI innovation is not impeded. Below we offer three examples of tangible federal

⁶ "Nondiscrimination in Health Programs and Activities," 89 Fed. Reg. 37522 (May 6, 2024)

legislative changes that could be made to enable AI-assisted care to reach American patients:

- Amend Section 5102(b) of the Deficit Reduction Act (DRA). Congress enacted Section 5102(b), also known as the “OPPS cap” in 2007. The law was created to address concerns regarding rapid growth in federal spending on imaging services, particularly for advanced imaging modalities—such as computed tomography (CT), magnetic resonance imaging (MRI), and nuclear medicine—as compared to growth in spending among less advanced imaging modalities, such as x-ray or ultrasound. AI services were not contemplated at the time of enactment as within the scope of the OPPS cap. Recently, CMS has begun to add AI services which analyze and provide diagnostic interpretations of images to the OPPS cap list. These services are not imaging services as contemplated by Congress when Section 5102(b) was enacted. AI healthcare services should not be on the OPPS cap list and Section 5102(b) should be amended to exclude such services. While this cap was originally created to control spending, it now poses a barrier to the adoption of technologies that can improve diagnostic accuracy and reduce unnecessary care.
- Review the Mammography Quality Standards Act (MQSA). The MQSA was enacted in 1994 and requires that all mammography facilities (except facilities of the Department of Veterans Affairs (VA)) be accredited by an approved accreditation body and certified by the FDA (or an approved State certification agency). The MQSA requires that “mammograms be interpreted by a physician who is certified as qualified to interpret radiological procedures, including mammography.” There is a concern that this language may preclude the use of AI analysis to interpret breast imaging for the identification of cancer, which is a front line use case for AI. When Congress passed the MQSA in 1994, the possibility of FDA-authorized, AI-enabled mammogram interpretation was not contemplated, and the existing statutory language hinders the development of AI technology that would advance Congress’ goal in passing the MQSA of high quality care for patients. To ensure patient access to advanced breast cancer detection technology, changes to the MQSA that will allow for the adoption of AI to enhance screening and early detection should be reviewed and considered.
- Amend the Social Security Act to Allow Glaucoma Screening. Section 1861(uu) of the Social Security Act (SSA) requires that glaucoma screening be furnished by “by or under the direct supervision of an optometrist or ophthalmologist who is legally authorized to furnish such services under State law (or the State regulatory mechanism provided by State law) of the State in which the services are furnished, as would otherwise be covered if furnished by a physician or as an incident to a physician’s professional service...”. Like in many other parts of the SSA, this

requirement precludes the use of fully autonomous AI services to perform an essential Medicare benefit – in this case, glaucoma screening. the SSA should be amended to allow for the use of rigorously validated, FDA-authorized autonomous AI services for glaucoma screening and the provision of other healthcare services.

III. Conclusion

Your efforts to develop an AI Action Plan coincide with a time of great excitement for health AI. This administration is in a unique position to usher in a new era of innovation, efficiency, and improved patient health outcomes through the rapid development of health AI technologies.

We look forward to working with White House OSTP and the Networking and Information Technology Research and Development (NITRD) National Coordination Office (NCO) to ensure that patients and providers have access to FDA-authorized, rigorously validated healthcare AI. If you have any questions about our comments, please contact me at

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Kind regards,

[REDACTED]

Cybil Roehrenbeck
Executive Director
AI Healthcare Coalition

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